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EXECUTIVE CORE QUALIFICATION NARRATIVES

Challenge • Context • Action • Result (CCAR) | Prepared for agency and QRB interviews

How to use: Under OPM's 2025 SES guidance, ECQ narratives are not submitted with the application; the two-page resume is the application and the ECQs are assessed through structured interviews. OPM allows candidates to refer to an outline during the QRB interview but discourages reading from a script. Use these narratives to prepare, then reduce each one to key points.

ECQ 1 COMMITMENT TO THE RULE OF LAW AND THE PRINCIPLES OF THE AMERICAN FOUNDING

Sub-competencies: *Knowledge of the American System of Government* • *Commitment to the Rule of Law* • *Civic-Mindedness*

Building a lawful, trusted investigations program for 52 VA facilities (2023–2025)

CHALLENGE

When I became Director of Western Region Investigations in February 2023, I was responsible for investigating care quality, patient safety, and whistleblower disclosures at 52 VA facilities in six western VISNs. My challenge was to make every investigation fair, legally sound, and able to stand up to Inspector General and congressional review, while protecting the employees who came forward.

CONTEXT

Many of our cases were whistleblower disclosures referred to VA by the Office of Special Counsel, which come with statutory reporting requirements and strict protections against retaliation. Review methods differed from team to team, some facilities challenged our findings as inconsistent, and congressional offices were following several cases closely on behalf of Veterans and their families.

ACTION

- **First,** I standardized how every investigation applied Title 38, VHA directives, and whistleblower protection law, with a single evidence standard and a legal and clinical review before any report left my office.
- **Second,** I set safeguards to protect whistleblower identities and to flag any sign of retaliation to leadership immediately, so employees could report problems without fear.
- **Third,** I tracked every Office of Special Counsel referral against its statutory deadline and personally reviewed each report before it was sent.
- **Fourth,** I briefed congressional staff and VA leaders on systemic findings, explaining what the law and VA policy required and how each recommendation served Veterans.

RESULT

- Led 140+ investigations that issued 600+ recommendations, with 94% concurrence from VA leadership.
- Every report met its statutory deadline and withstood Inspector General and congressional scrutiny with no findings reversed.
- The behavioral health investigation track I created was adopted nationally.

ECQ 2 DRIVING EFFICIENCY

Sub-competencies: *Fiscal Responsibility* • *Managing Resources* • *Leveraging Technology*

Redesigning national corrective-action tracking on a $22M budget (2025–2026)

CHALLENGE

When I took over Clinical Quality Review in October 2025, recommendations from our reviews took an average of 410 days to close, and only 52% closed on time. I had to make the program faster and more effective without new funding.

CONTEXT

The office runs on a $22M budget and about 95 reviewers and analysts. Three regional teams had used different tracking spreadsheets and contracts, so the same facility could be visited twice while high-risk sites waited, and leaders could not see which fixes were overdue. Unresolved recommendations meant known risks to Veterans stayed open.

ACTION

- **Fiscal responsibility:** I reviewed every line of the budget and about $6M in analytics and review-support contracts, ending duplicate work and tying each contract to measurable deliverables.
- **Managing resources:** I moved reviewers from routine rotation visits to risk-based reviews, focusing staff on the 25 highest-risk facilities instead of spreading them evenly across all sites.
- **Leveraging technology:** I replaced regional spreadsheets with one national tracking system that shows every open recommendation, its owner, and its due date, and I ensured it met VA privacy and security requirements before launch.

RESULT

- Cut average time to close recommendations from 410 days to 180 within one year.
- Raised on-time closure from 52% to 88%.
- Delivered these gains within the existing $22M budget, with no new positions.

ECQ 3 MERIT AND COMPETENCE

Sub-competencies: *Technical Skill* • *Problem Solving* • *Agility and Resilience*

Building VHA's predictive risk model to target quality reviews (2025–2026)

CHALLENGE

I needed to find the VA medical centers most likely to have serious quality and safety problems before Veterans were harmed, rather than waiting for adverse events to surface them.

CONTEXT

Reviews had been scheduled by calendar rotation, so serious problems were often found only after a death or a whistleblower complaint. The data that could predict risk sat in separate systems: patient safety reports, suicide-risk indicators, and staffing levels. Leaders were skeptical that a model could be trusted to decide where reviewers went.

ACTION

- **Applied expertise:** Drawing on my clinical background as an LCSW and my CPHQ quality training, I defined which indicators actually signal risk to Veterans, especially in behavioral health.
- **Solved the problem with data:** I led analysts in combining the three data sources into one risk score for every medical center, then tested it against past adverse events to prove it worked before using it.
- **Adapted when obstacles arose:** When staffing data from several VISNs proved incomplete, I added a manual verification step instead of delaying launch, and refined the model each quarter as better data arrived.

RESULT

- Directed reviews to the 25 highest-risk facilities nationwide.
- Risk-targeted reviews found serious safety issues earlier, feeding directly into the corrective-action system and the behavioral health quality initiative.
- The model became the standard method for scheduling national quality reviews.

ECQ 4 LEADING PEOPLE

Sub-competencies: *Accountability • Developing Others • Executive Judgment*

Uniting three regional teams into one national office (2025–2026)

CHALLENGE

I had to merge three regional review teams, about 95 reviewers and analysts, into one national Clinical Quality Review office without losing talent or momentum.

CONTEXT

Each team had its own culture, protocols, and leaders, and many staff worried about losing their roles or autonomy. Review quality varied by region, there was no common standard for what a strong reviewer looked like, and performance problems had been left unaddressed in some units.

ACTION

- **Set clear expectations:** I issued one set of review protocols, reporting templates, and escalation paths across all 18 VISNs, and wrote measurable quality and timeliness goals into every supervisor's performance plan.
- **Held people accountable:** I addressed persistent performance gaps directly through coaching and performance plans and reassigned work where results did not improve.
- **Developed others:** I built a reviewer competency model and certification track, mentored new supervisors, and served on SES and GS-15 selection panels to bring strong leaders into the agency.
- **Used executive judgment:** I established and chair the National Review Board, so senior leaders decide high-risk findings together and every finding has an accountable owner.

RESULT

- Completed the consolidation into one national office with standard practices across 18 VISNs.
- Promoted 12 internal staff into lead roles through the certification track.
- Supported on-time recommendation closure rising from 52% to 88%.

ECQ 5 ACHIEVING RESULTS

Sub-competencies: *Operational Mindset • Innovation • Strategic Thinking*

Closing suicide-risk screening gaps across 40+ VA sites (2025–2026)

CHALLENGE

VHA leadership made suicide prevention a top priority, but our reviews kept finding gaps in suicide-risk screening and safety planning. I was asked to lead the effort to close those gaps nationwide.

CONTEXT

Responsibility was split among the Office of Mental Health and Suicide Prevention, 18 VISNs, and facility leaders, and no one owned the fixes from end to end. Findings from earlier reviews had been issued but often not acted on, and the stakes were Veterans' lives.

ACTION

- **Strategic thinking:** I aligned our review findings with VHA's suicide-prevention goals and identified the 40+ sites with the largest screening and safety-planning gaps.
- **Operational mindset:** As lead integrator, I brought the program office, VISN leaders, and facility leaders together, assigned an owner and deadline to every gap, and tracked progress through the National Review Board.
- **Innovation:** I turned recurring findings into proposed national policy instead of facility-by-facility fixes, and briefed the Under Secretary for Health and congressional staff more than 20 times to build support.

RESULT

- Closed suicide-risk screening and safety-planning gaps at 40+ sites.
- Findings led to three new VHA directives on behavioral health safety.
- Earned the VHA Under Secretary for Health Award for Quality Improvement.